

SEVERITY**HIGH**REPORT ID
CONFIDENCE**EMERGENCY OVERRIDE**

CRITICAL — Life-threatening findings

TRIGGERED REASONS

- SpO2 critically low (88%) — respiratory failure risk
- Heart rate elevated to 112 bpm — possible sepsis
- Fever 103°F with rigors in elderly diabetic

IMMEDIATE FIRST AID

- Position patient upright to ease breathing
- Administer oxygen via face mask at 6-8 L/min if available
- Give paracetamol 1g orally for fever
- Do NOT give food or drink — preparation for possible procedures

Immediate ambulance transport to district hospital with oxygen support.**REFERRAL LEVEL HIGH****Facility** District Hospital with ICU**Urgency** Immediate (within 1 hour)**Transport** Ambulance with oxygen and IV setup. If ambulance unavailable, private vehicle with portable oxygen cylinder and accompanying health worker.**RECOMMENDATION**

Admit to ICU. Start empirical antibiotics, oxygen therapy, IV fluids, and investigations (chest X-ray, CBC, BMP, blood culture, sputum culture).

TRANSFER CHECKLIST

- Patient identity card and Aadhaar
- Previous medical records and current medication list
- Latest vitals chart with trend
- Voice/clinical notes from CHW
- Emergency contact numbers of family
- Allergy information card
- Insulin and diabetic medication supply for 24 hours
- Cash for hospital admission deposit

PATIENT INFORMATION

Patient Name	—
Age	—
Gender	—
Phone	—
Address	—

HIGH

Severity: HIGH — Urgent

Confidence: HIGH

Elderly diabetic patient with 3-day fever, productive cough, pleuritic chest pain, and SpO2 of 88%. Suspicion of community-acquired pneumonia with possible sepsis. Immediate referral to district hospital with oxygen support and empirical antibiotics is indicated. Vitals show tachycardia, low-grade fever, and borderline hypotension.

PATIENT VITALS

PARAMETER	VALUE	UNIT	STATUS
Blood Pressure	—	mmHg	—
Heart Rate	—	bpm	—
Temperature	—	°C	—
Oxygen Saturation (SpO2)	—	%	—
Blood Glucose	—	mg/dL	—

ANOMALY FINDINGS

- Critical: SpO2 below 90% (88%)
- Tachycardia: 112 bpm
- Fever above 102°F (103°F)
- Diabetic with elevated glucose (186 mg/dL)
- Pleural-type chest pain with productive cough
- Reduced urine output in last 24 hours

LABORATORY FINDINGS

No laboratory values recorded.

LAB ALERTS

- Elevated WBC (14,800) — infection
- Elevated CRP (186 mg/L) — severe inflammation
- Elevated procalcitonin (1.8 ng/mL) — bacterial infection likely
- Reduced eGFR (42) — CKD worsening
- Elevated D-dimer (1.2) — needs PE workup

CLINICAL SUMMARY

Elderly diabetic patient with 3-day fever, productive cough, pleuritic chest pain, and SpO2 of 88%. Suspicion of community-acquired pneumonia with possible sepsis. Immediate referral to district hospital with oxygen support and empirical antibiotics is indicated. Vitals show tachycardia, low-grade fever, and borderline hypotension.

VOICE TRANSCRIPT / SYMPTOMS NOTES

[illegible]

OCR EXTRACTED TEXT

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AI Disclaimer — This report is generated using AI-assisted triage support and should not replace professional clinical judgment. Final diagnosis and treatment decisions remain the responsibility of licensed healthcare professionals.